Response to HHS Request for Information:
Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Submitted by:
Karina Lupercio
Founder & Principal Consultant
Lumenex Advisory LLC

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Introduction

I am submitting this response based on more than two decades of experience operating within the U.S. healthcare ecosystem, with a focus on healthcare operations, data integration, payment models, and technology implementation. My perspective is grounded less in theoretical AI capability and more in the realities of how clinical, administrative, and financial systems function together in practice.

While artificial intelligence holds significant promise for improving clinical outcomes, reducing administrative burden, and lowering costs, adoption often stalls not because of model performance, but because of structural, financial, and organizational friction. My comments below focus on those friction points, specifically as they relate to private sector innovation and reimbursement alignment.

Response to Question 1:
What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

From an operational standpoint, the most significant barriers to AI adoption in clinical care are not technical. They are organizational, administrative, and incentive-based.

Ownership and accountability for adoption are often unclear. AI initiatives are frequently treated as IT implementations, when in reality they require coordinated change across clinical, operational, compliance, and administrative teams. When responsibility for workflow redesign and ongoing maintenance is diffused, adoption stalls even when the technology itself performs as intended.

A recurring barrier is resistance driven by perceived increases in workload. In my experience implementing large-scale provider data management systems prior to the availability of modern AI tools, downstream teams often resisted adoption because new workflows were viewed as "extra steps." The long-term value of structured data capture, namely, the ability to generate reliable, high-quality reporting, was not immediately visible to end users. While leadership often desired improved analytics and insights, insufficient attention was paid to explaining how those outcomes depended on consistent, incremental changes in daily workflows.

Over time, these steps typically became easier and more efficient, but early resistance significantly slowed adoption. AI magnifies this dynamic: without deliberate workflow design, education, and change management, AI tools risk being perceived as additive burden rather than enabling infrastructure.

The most common point of failure is the handoff to end users. When cultural transformation and workflow ownership are not addressed explicitly, even well-designed systems fail to achieve sustained use. This is not a technology failure, but an implementation and governance one.

Response to Question 2:
What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

Misaligned incentives and uncertainty around value recognition remain significant obstacles to AI adoption.

Payment models rarely account for the trust, education, and governance work required for adoption. Many AI tools deliver value by improving efficiency, reducing administrative burden, or augmenting clinician decision-making rather than producing discrete billable events. As a result, organizations often struggle to justify investment when reimbursement pathways are unclear or nonexistent.

Even relatively narrow AI tools face adoption hurdles when trust is not addressed. For example, implementing AI-enabled note-taking tools in healthcare settings frequently raises concerns about privacy, recording, and data usage. Despite conducting due diligence to ensure SOC 2, HIPAA, and GDPR compliance, adoption was initially limited until clear guardrails were established around how, when, and for what purpose the tools would be used. Education and transparency were essential to building confidence and enabling responsible use.

HHS could accelerate adoption by aligning reimbursement and program design with real-world implementation costs. Incentives that recognize not only clinical outcomes but also the operational effort required to safely deploy AI, including training, governance, and workflow integration, would reduce adoption risk. Pilot programs, demonstration models, or value-based payment structures that explicitly accommodate AI-enabled efficiency gains would encourage experimentation while generating evidence to inform broader policy.

Conclusion

AI adoption in clinical care will not accelerate through technical innovation alone. It requires alignment across organizational governance, administrative capacity, and financial incentives. By focusing on incentive alignment, reimbursement clarity, and reduction of administrative friction, HHS has an opportunity to enable meaningful, responsible adoption that delivers real value to patients, clinicians, and the healthcare system as a whole.